

C01 HEALTH AND WELL-BEING PROMOTION | P

Intent : Promote a deeper occupant understanding of the WELL features pursued by the project or organization and how building design, operations and policies impact health and well-being.

Summary : This WELL feature requires projects to provide a guide to occupants that highlights the WELL features pursued by the project, the relationship between health and buildings and available health resources and programs.

Issue : Health literacy refers to a person's cognitive and social ability to access, interpret and understand basic health information, as well as the ability to act on that understanding to maintain health.¹⁻⁴ Health literacy is influenced by sociocultural factors, such as age, socioeconomic status, mental health, cultural background, language and communication abilities, prior health experiences and how healthcare delivery and education systems deliver care, health information and health education.¹ Low health literacy is linked to lower use of preventive care (e.g., flu shots), poor management of chronic conditions (e.g., high blood pressure) and lower self-reported mental and physical health.^{5,6} Studies estimate that low health literacy costs the U.S. economy anywhere between \$70-240 billion each year.^{5,7}

Solutions : Multi-modal programming, educational materials and communications can help promote health literacy and increase positive health outcomes.^{8,9} By supporting awareness of health and wellness programs and policies, projects can promote health literacy and encourage engagement with health resources, leading to both individual benefits - like increased participation in healthy behaviors and use of health services - and also employer benefits, providing an estimated 4:1 return on investment.^{5,10}

PART 1 PROVIDE WELL FEATURE GUIDE

For All Spaces:

1: WELL feature guide

A physical or digital WELL feature guide, such as the WELL report, will be prominently displayed and/or made widely available to all occupants upon certification achievement or completion of a review cycle, meeting the following requirements:

- a. Describes the WELL features achieved by the project or organization.

2: Communications

The following requirement is met:

- a. Quarterly communications (e.g., emails, modules, trainings) are sent to regular occupants, and onboarding communications are given to new employees (as applicable), about health resources, programs, amenities and policies available to them addressed by the WELL features achieved by the project or organization.

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C02 INTEGRATIVE DESIGN | P

Intent : Facilitate a collaborative project process and support adherence to collective well-being and sustainability goals.

Summary : This WELL feature requires project teams to facilitate an inclusive and collaborative planning and orientation process and to establish a health-oriented mission.

Issue : Projects that do not consult stakeholders during planning and development processes often do not serve stakeholder needs and may even negatively impact the health and well-being of certain populations.^{1,2} In particular, studies show that low-income and minority populations are most often excluded from planning discussions and decision-making, leading to developments that do not address their needs and may even increase their exposure to a disproportionate burden of poor health conditions, displacement, pollution, crime and lack of access to opportunities and services.^{1,2}

Solutions : Engaging diverse stakeholders from the project onset creates the opportunity for collaborative dialogue between key decision-makers, planners and individuals who will be impacted by the project. Collecting stakeholder input can help a project identify and address its essential goals for health promotion and incorporate design that best celebrates the project's unique identity, culture and place, creating a space that meets the needs of all stakeholders and enriches the well-being of both occupants and visitors.^{1,3,4} Establishing a health-centered mission and orienting stakeholders to how the project will adhere to that mission through design and operations, including WELL features, can help individuals remain engaged in the space and empower them to utilize all available health and wellness programs and policies.⁵

PART 1 FACILITATE STAKEHOLDER CHARRETTE

For All Spaces:

1: Stakeholder charrette

Early in the planning process for the pursuit of a WELL designation, representatives from the organization or project team (e.g., leadership, human resources, project managers) facilitate a collaborative discussion that meets the following requirements:

- a. Includes representative key stakeholders including (as applicable):
 1. Owner.
 2. Facilities manager.
 3. Architects and engineers.
 4. Contractors.
 5. Employees or other occupants.
 6. Community members (if the project or organization has substantial impact on the surrounding community).
- b. Identifies the health and well-being needs of your occupants.
- c. Identifies your organization's objectives for health promotion.
- d. Defines goals, related to each of the following:
 1. Occupant health and well-being.
 2. Resilience.
 3. Energy and resource management.
 4. Protection of water resources and ecosystems.
 5. Healthy and sustainable material cycles.
 6. Community inclusion.
 7. Clean and healthful local environments.

2: Stakeholder orientation

Tours of the space, communicating existing building operations, maintenance, programs and policies support adherence to WELL requirements, are conducted and made available to the following groups:

- a. All stakeholders in the development process, including (as applicable) the owner, manager, facilities management team, architects, engineers, existing employees, occupants, residents, contractors and community members.
- b. New employees during onboarding.

PART 2 PROMOTE HEALTH-ORIENTED MISSION

For All Spaces:

The project or organization establishes a health-oriented mission that meets the following requirements:

- a. Outlines the project's or organization's objectives for health promotion.⁵
- b. Includes a statement about supporting and improving occupant health.⁵
- c. Incorporates relevant organizational goals or strategies established during the stakeholder charrette.
- d. Is made available to all occupants.

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C03 EMERGENCY PREPAREDNESS | P

Intent : Enable organizations, families and individuals to prepare and respond to diverse emergency situations.

Summary : This WELL feature requires projects to undertake a risk assessment, create an emergency management plan for natural, human-caused, technological and health-related emergencies and educate occupants on the plan to support emergency preparedness and response.

Issue : Natural disasters kill around 90,000 people and affect close to 160 million people worldwide every year, with both an immediate and long-term impact on human lives and built spaces.¹ Older adults, individuals with disabilities, pregnant women and children may have special needs during an emergency and are particularly vulnerable when disaster strikes.¹ The U.S. Federal Emergency Management Agency estimates about 40–60% of small businesses permanently close following a disaster due to the lack of a comprehensive disaster preparedness plan with proper mitigation strategies.² Beyond natural disasters, infectious disease epidemics have increased in the 21st century, involving rapid global spread due to travel, trade and urbanization.^{3,4} The COVID-19 pandemic infected six million people and spread to almost every continent within five months, impacting the social and economic livelihood of the global population on an incalculable scale.⁵

Solutions : An effective emergency management plan requires an understanding of potential local hazards, the needs of vulnerable groups, the responsibilities of the emergency response team and building response capabilities.⁶ Emergency management plans, including risk assessments, occupant drills and enhanced emergency communications, can help organizations be better prepared to handle unforeseen events, minimize occupant confusion and improve coordination and safety during emergency situations.^{3,7–11} Robust emergency preparedness and response measures can also help to slow the spread of infectious disease and minimize secondary mortality.¹²

PART 1 DEVELOP EMERGENCY PREPAREDNESS PLAN

For All Spaces:

The following requirements are met:

- a. A risk assessment is undertaken to address at minimum the following:
 1. Identify project assets (e.g., employees, facilities).
 2. Establish a process for occupants or groups who may be more vulnerable (e.g., older adults, people with disabilities, pregnant women, children) to confidentially identify specific needs they may have during an emergency.¹³
 3. Evaluate potential impacts of relevant hazards and identify high-risk hazards.
 4. Determine emergency management planning priorities.
- b. An emergency management plan is in place outlining response in the case of emergency situations within the building or surrounding community, addressing at minimum the following hazards:
 1. Natural (e.g., flood, tsunami, wildfire, earthquake, heatwave).
 2. Fire.
 3. Health (e.g., acute medical emergency, infectious disease pandemic).
 4. Technological (e.g., power loss, chemical spill, explosion).
 5. Human-caused (e.g., civil unrest, active shooter, terrorism).
- c. The emergency management plan meets the following requirements:
 1. Incorporates annual (at minimum) inventory and maintenance of building emergency response resources (e.g., first aid kits, automated external defibrillators (AEDs), emergency notification system, personal protective equipment) and operations capabilities (e.g., backup power, remote management systems).
 2. Includes a list of specialized personnel that is updated annually (at minimum) and includes roles and contact information of the emergency response team.¹³
 3. Plan is reviewed and updated (as needed) on an annual basis and is easily accessible to all regular occupants.
- d. Regular occupants are provided education and training on emergency preparedness and response, including the following:
 1. Communications about the emergency management plan and related resources, including guidance by relevant local-, state-, regional- or global-level emergency response agencies (e.g., WHO, government emergency management agency or equivalent), annually (at minimum), to employees during new employee onboarding and during an emergency event.
 2. Practice drills or other operations-based or discussion-based exercises conducted annually (at minimum) for each high-risk hazard identified in the risk assessment, and conducted every two years (at minimum) for other hazards covered under the emergency management plan.¹³

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C04 OCCUPANT SURVEY | P

Intent : Evaluate the experience and self-reported health and well-being of building users through occupant surveys.

Summary : This WELL feature requires projects to collect feedback from building users through third-party or custom surveys on their health, well-being and satisfaction with their environment, particularly on topics related to WELL strategies.

Issue : Given the wide diversity in the design, operation and use of built spaces, it is difficult to gauge which design, policy and programmatic approaches will benefit the health and well-being of the most individuals in a space.^{1,2} For example, decision-makers and users of the space often experience things differently.¹ Employers do not often put methods in place to systematically gather input on the experience of their employees, such as satisfaction with policies, design and maintenance or feelings of overall health.^{3,4,5}

Solutions : Surveys are an established tool for understanding and evaluating people's perceptions of indoor environmental conditions, wellness policies and personal health and well-being.⁶⁻⁸ Psychometrically validated surveys and questions ensure that sensitive questions are framed appropriately and measure what they are intended to measure, and when combined with environmental satisfaction questions can effectively capture high-quality data.⁹⁻¹² Employees who perceive that their employer acts on their feedback are four times more likely to stay with their company; moreover, investing in the employee experience can reduce turnover and absenteeism and increase productivity, retention and engagement.^{3,5} Surveys that ask building users about their satisfaction with indoor environmental quality and workplace wellness amenities and policies help evaluate the effectiveness of existing health and wellness interventions, identify opportunities to create a healthier environment and bring employers significant returns on investment.^{5,12-14}

PART 1 SELECT PROJECT SURVEY

For All Spaces:

Option 1: Pre-approved survey

For projects with ten or more eligible employees, the following requirement is met:

- a. A survey is selected from a survey provider listed on [Reference](#).

OR

Option 2: Custom survey

For projects with ten or more eligible employees, the following requirement is met:

- a. A survey is created that covers the topics listed in Appendix C1.

OR

Option 3: Small employee population

The following requirement is met:

- a. There are fewer than 10 eligible employees in this project.

PART 2 ADMINISTER ANNUAL SURVEY AND REPORT RESULTS

For All Spaces:

1: Survey administration

The following requirements are met:

- a.

All eligible employees are invited to participate in the survey annually. Regular reminders are sent to eligible employees to complete the survey.

- b.

Survey protects all participant-identifying data through appropriate measures such as anonymous reporting and safe data storage. Any communication of results should be on an aggregated basis, such that no participant can be identified.

- c.

Analysis of responses is conducted by a qualified survey professional.

2: Result reporting

The project or organization annually submits, through the platform, the following:

- a. Project and survey data, including:

1. Total number of employees invited to complete the survey and number of employees who completed the survey.
2. Date survey started and finished.
3. Project location.
4. Project type.
5. Level of WELL achievement, if applicable (e.g., WELL Health-Safety Rated).

- b.

Aggregated, anonymized survey results.

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C05 ENHANCED OCCUPANT SURVEY | O (MAX : 4 PT)

Intent : Build on minimum occupant survey requirements with enhanced and customized questions to comprehensively evaluate and respond to the experience and self-reported health and well-being of building users.

Summary : This WELL feature requires projects to build on minimum occupant survey requirements by collecting and responding to more in-depth and customized information from building users on their health, well-being and satisfaction with their environment including topics related to WELL strategies, both before and during occupancy.

Issue : Given the wide diversity in the design, operation and use of built spaces, it's difficult to gauge which design, policy and programmatic approaches will benefit the health and well-being of the most individuals in a space.^{1,2} For example, decision-makers and users of the space often experience things differently.¹ Employers do not often put methods in place to systematically gather input on the experience of their employee, such as satisfaction with policies, design and maintenance or overall health.^{3,4,5}

Solutions : Occupancy surveys measure the extent to which a building promotes user health and comfort.^{6-8,9-11} Specifically, psychometrically validated surveys evaluate building users' experience, frame sensitive questions appropriately and measure what they are intended to measure.^{12,13} Incorporating a range of survey topics, and utilizing both pre- and post-occupancy surveys, provides a comprehensive picture of which interventions impact building users' satisfaction.^{11,14,15} Moreover, interviews and focus groups provide key insights not captured in surveys.^{16,17,18} Stakeholders can use results to identify priority interventions to make spaces healthier and more productive.¹⁹ Offering the opportunity to provide feedback, along with an action plan to address dissatisfaction, can improve employee morale and retention while creating a healthier environment for all.^{5,20,21,22-25}

PART 1 UTILIZE ENHANCED SURVEY (MAX : 1 PT)

For All Spaces:

1: Enhanced survey administration

For projects with ten or more eligible employees, the following requirements are met:

- a. Meet Feature C04 Part 1 using a third-party or pre-approved survey provider.
- b. Address at least one of the topics listed in Appendix C2 through a minimum of three additional survey questions utilizing a pre-approved survey provider listed on [Reference](#).

2: Result analysis and reporting

The project or organization meets the following requirements:

- a. Based on survey results, investigate correlations, inferential statistics (such as multivariate analysis), or other analyses beyond descriptive statistics.
- b. Submit the following through the platform annually:
 1. Aggregated, anonymized survey results for the additional topics selected from Appendix C2.
 2. Results of enhanced analysis.

PART 2 UTILIZE PRE- AND POST-OCCUPANCY SURVEY (MAX : 1 PT)

For All Spaces:

1: Baseline and annual survey administration

The project or organization meets the following requirement:

- a. Prior to achieving a WELL milestone, administer a survey for eligible employees using the same [Reference](#) that will be used for Feature C04.

2: Result analysis and reporting

The project or organization meets the following requirements:

- a. Compare results from the baseline survey against subsequent survey results.
- b. Submit aggregated, anonymized survey results through the platform on the following:
 1. Aggregated, anonymized results of the baseline survey.
 2. Comparison between the results of the baseline and annual surveys.
 3. Total number of employees invited to complete the survey and number of employees who completed the survey, each time it was administered.
 4. Date each survey started and finished.
 5. Location where each survey was administered.
 6. Project type.
 7. Level of WELL achievement, if applicable
 8. Sociodemographic information (age and gender at a minimum).

Note :

Additional baseline survey is not required at recertification.

PART 3 IMPLEMENT ACTION PLAN (MAX : 1 PT)

For All Spaces:

1: Satisfaction Targets

The project or organization creates and implements a plan that addresses the following:

- a. Defines target satisfaction levels reported in the annual survey.

2: Improvement Strategies

The project or organization submits on an annual basis:

- a. Comparison of satisfaction results to aspirational targets.
- b. Improvement strategies to be implemented, if applicable.

PART 4 FACILITATE INTERVIEWS, FOCUS GROUPS AND/OR OBSERVATIONS (MAX : 1 PT)

For All Spaces:

1: Administration of interviews, focus groups and/or observations

The project or organization annually conducts "evaluations" (defined here as stakeholder interviews, focus groups and/or observations) that meet the following requirements:

- a. Are conducted and analyzed by a professional experienced in qualitative research.
- b. Comprise a culturally representative sample of the population.
- c. Discuss the impact that the built environment and organizational initiatives have on occupant health and well-being.
- d. Protect participant privacy and identity.

2: Result analysis and reporting

The project or organization meets the following requirements:

- a. Compare results from the evaluations to the survey results, if applicable.
- b. Annually submit aggregated, anonymized results of the evaluations through the platform on the following:
 1. Comparison between the results of the evaluations and the survey results, as applicable.
 2. Total number of employees and number of employees who participated in the evaluations.
 3. Date the evaluations started and finished.
 4. Project location(s).
 5. Project type(s).
 6. Level of WELL achievement, if applicable.
 7. Sociodemographic information of participants (age and gender at a minimum).

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C06 HEALTH SERVICES AND BENEFITS | O (MAX : 5 PT)

Intent : Support the overall health and well-being of individuals and their families by offering comprehensive health benefits, policies and services.

Summary : This WELL feature requires projects to provide access to essential and on-demand health services, paid sick leave and immunizations.

Issue : Access to basic healthcare services is one of five key pillars that form the social determinants of health.¹ Access includes physical or geographic access, affordability, and quality or acceptability of care, and access varies based on race, ethnicity, socioeconomic status, age, sex, disability status, sexual orientation, gender identity and location.²⁻⁵ Unvaccinated individuals pose a risk to public health, and seasonal flu causes severe illness and death in high-risk populations, costing the \$10.4 billion in annual healthcare costs; the average hospitalized flu case in Canada costs \$11,092.⁶⁻⁹ Delays between identifying a need for care and receiving services can increase complications, costs and hospitalization.^{4,10} Moreover, while 94% of the world's countries mandate paid sick leave, the U.S. and Korea are the only OECD countries that do not, and 40% of American employees have no sick leave.^{11,12} Studies estimate that 20 million Americans and 37% of employees in the United Kingdom go to work sick because they lack sick leave or have only one-day sick leave, respectively, infecting colleagues as a result.^{13,14} Employees may also go into work when sick if their sick leave does not offer sufficient wage replacement.¹³

Solutions : Basic essential healthcare services include medical, dental, vision, mental health, substance use, preventive screenings, disease management and biometric assessments.⁴ Providing free on-site flu vaccines with education on good health habits can increase vaccination rates and reduce flu cases.¹⁵ Providing timely access to health services can relieve both actual and perceived barriers to care.^{4,16} Studies demonstrate that the overwhelming majority of employees seek one-on-one benefits consultation and flexible coverage options so they can opt into coverage that best meets their individual schedule and health needs.¹⁷ Studies also show that implementing paid sick leave reduces contagion in the workplace, improves employee productivity and reduces employee turnover.^{13,14,18-20} Overall, enhancing access to essential healthcare and paid sick leave can help improve the physical, social and mental health of individuals and communities.^{2,4}

PART 1 PROMOTE HEALTH BENEFITS (MAX : 1 PT)

For All Spaces:

A health benefits policy meets the following requirements:

- a. Is available to all eligible employees and their designated dependents (e.g., spouse, domestic partner, child) at no cost or subsidized that includes the following services:
 1. Medical care.
 2. Dental care.
 3. Vision care.
 4. Sexual and reproductive health services, including obstetrics and gynecology (OB-GYN) services and sexually transmitted infection (STI) testing and treatment.
 5. Medication/prescription coverage.
 6. Essential immunizations, as determined based on region.
 7. Preventive screenings and biometric assessments.
 8. Tobacco cessation programs.
 9. Infectious disease testing (e.g., tuberculosis, malaria, COVID-19) during a regional or global infectious disease outbreak, epidemic or pandemic as declared by a regional or global public health agency (e.g., WHO, disease control and prevention centers or equivalent).
- b. Confidential benefits consultations are available with clearly identified and qualified support staff (e.g., benefits counselor, human resources representative).

PART 2 OFFER ON-DEMAND HEALTH SERVICES (MAX : 1 PT)

For All Spaces:

A health benefits policy is available for all eligible employees that provides health services at no cost or subsidized, on-site and in-person within 400 m of the project boundary or through a telemedicine provider or digital health platform. The health services program meets the following requirements:

- a. Experienced and qualified healthcare providers (e.g., physician, nurse practitioner, physician assistant) are available to provide confidential medical treatment for episodic, recurrent, urgent or other illnesses before, during and/or after regular business hours.
- b. A scheduling system that allows drop-ins and/or appointment booking.
- c. Eligible employees are permitted to use services during the workday if appointments are only available during regular business hours.

PART 3 OFFER SICK LEAVE (MAX : 1 PT)

For All Spaces:

A sick leave policy that meets the following requirements is available to all eligible employees:

- a. Leave is offered upfront or accrued for use during any 12-month period for any health condition and meets one of the following requirements:
 1. Short-term sick leave for all eligible employees, distinct from paid time off and family leave, at least 10 days of which are paid at 50% or higher of the employee's full salary or wages.
 2. At least 20 days of combined paid time off and sick leave, which are paid at 50% or higher of the employee's

full salary or wages. Projects using a blended policy are not eligible to pursue Feature M06 Part 1.

- b. Statement that discourages employees from coming into work when they feel sick and from doing work while on sick leave.
- c. At least one of the following:
 - 1. At least 12 weeks of sick leave (which may be unpaid) during any 12-month period for a chronic or serious health condition that involves inpatient care in a hospice or residential healthcare facility (e.g., stroke, infectious disease, surgery) or a health condition that requires continuing treatment and/or supervision by a healthcare provider (e.g., diabetes, asthma, cancer).
 - 2. Part-time options, flexible schedules or permission to work from home when recovering from serious health conditions.

PART 4 SUPPORT COMMUNITY IMMUNITY (MAX : 1 PT)

For All Spaces:

The project identifies an immunization relevant to the target population and implements an immunization program which includes the following:

- a. Makes the immunization available to regular occupants on at least an annual basis at no cost through either:
 - 1. An on-site clinic or program.¹⁵
 - 2. An off-site clinic or program (e.g., free community clinic, access through health care providers) and, for employees (as applicable), paid time during the workday to receive the immunization.²¹
- b. For employees, as applicable, at least one day of paid leave for recovery or sick leave following immunization.
- c. A campaign that addresses the following:¹⁵
 - 1. Provides regular occupants information on how the project facilitates immunization availability.
 - 2. Encourages or incentivizes, through monetary or non-monetary methods, regular occupants to receive the immunization.
 - 3. Educates regular occupants on the health reasons to receive the immunization.

PART 5 B PROVIDE ENHANCED HEALTH BENEFITS (MAX : 1 PT)

For All Spaces:

A health benefits policy is available for all eligible employees and their designated dependents (e.g., spouse, domestic partner, child) at no cost or subsidized. Health services must include at least four of the following:

- a. Nutrition support and services (e.g., medical nutrition therapy including nutrition supplements and enteral nutrition).²²⁻²⁴
- b. Complementary and integrative healthcare services (e.g., herbal therapy, mind and body practices such as acupuncture, massage, yoga, aquatic therapy).
- c. Non-Emergency Medical Transportation (NEMT) that includes reimbursable transportation both to and from medical appointments, utilizing a form of transportation that meets the medical needs of the individual and covering all associated expenses.²⁵
- d. Elder care, including home health care.
- e. Fertility Services (e.g., in vitro fertilization, iatrogenic infertility).
- f. Doula or other birth support workers.
- g. Comprehensive Abortion Care (CAC).
- h. Gender-affirming care including, at a minimum, hormone therapy and surgery.

Note :

This part is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

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C07 ENHANCED HEALTH AND WELL-BEING PROMOTION | O (MAX : 2 PT)

Intent : Cultivate a culture that prioritizes and promotes the health and well-being of all individuals.

Summary : This WELL feature requires projects to cultivate a culture of health through communications, health promotion programs and executive-level leadership.

Issue : Workplaces have the potential to promote and encourage healthy behaviors; however, if employees are unaware of the health promotion opportunities available, they are unlikely to participate.^{1,2} Moreover, studies of organizations with successful workplace wellness programs demonstrate that isolated incentives or programs, or even just health-promoting physical environments alone, will not be effective in promoting employee health or engaging employees in healthy behaviors unless they are part of an overarching culture of health integrated into everyday operations.¹

Solutions : The Robert Wood Johnson Foundation defines a culture of health as "a society that gives all individuals an equal opportunity to live the healthiest lives possible, whatever their ethnic, geographic, racial, socioeconomic or physical circumstances happen to be."³ Organizations can build a culture of health through customized health promotion programs that are integrated into operations and business strategy, promoted through consistent communications, championed by leadership at all levels and given dedicated resources.^{4,5} Incorporating incentives can raise employee participation and motivate behavior change such as weight loss and smoking cessation.⁶ Medical and absenteeism costs fall by about \$3.27 and \$2.73, respectively, for every dollar spent on workplace health programs.⁶ Successful health promotion programs can improve job satisfaction, sense of well-being, self-esteem and health status, and reduce stress and health risks.^{7,8} Organizational benefits include lower healthcare costs and absenteeism and improved productivity, recruitment, retention, culture and employee morale.^{7,8}

PART 1 PROMOTE CULTURE OF HEALTH (MAX : 1 PT)

For All Spaces:

1: Health promotion strategies

Occupant health and well-being is promoted through the following:

- a. Monthly digital communications to employees and/or regular occupants (as applicable) that address the following:
 1. Reinforce the project's culture of health.
 2. Market health promotion policies and programs.
 3. Highlight stories from regular occupants (as applicable) who exemplify the project's health culture.
 4. Offer education (e.g., tips and resources created by the project or a third party) on at least two topics within the ten WELL concepts.⁹
- b. Quarterly education sessions (e.g., workshops, lectures, seminars) that offer instruction on topics within the ten WELL concepts, covering at least two different concepts per year.

2: Health promotion coordinators

One of the following is present:

- a. Health promotion group that meets at least quarterly, is actively involved in planning and implementing health promotion programs and policies and seeks to cultivate a culture of health in the project.⁹
- b. Paid mid- or senior-level employee that plans and implements health promotion programs. Health promotion must be part of their job description, requirements and/or performance expectations.⁹ Projects that meet Part 2 Health Promotion Leader automatically fulfill this requirement.

PART 2 ESTABLISH HEALTH PROMOTION LEADER (MAX : 1 PT)

For All Spaces:

The following requirements are met:

- a. Project has at least one dedicated executive-level employee whose primary responsibility is to plan and oversee strategies that promote the physical, mental and emotional health and well-being of all employees (e.g., Chief Wellness Officer). The individual must be employed at the executive (C-Suite) level or report directly to a member of the executive (C-Suite) team.
- b. Executive-level employee's job description and performance expectations must include the following:
 1. Established metrics or KPIs for promoting organizational health and well-being that are linked to employee's performance evaluation.
 2. At minimum annual reports by the employee on the progress of health promotion strategies and employee engagement to the executive (C-suite) team, Board of Directors and/or equivalent high-level stakeholders.
- c. Executive-level employee is supported by at minimum one employee who helps plan and implement health promotion programs and policies.

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C08 NEW PARENT SUPPORT | O (MAX : 3 PT)

Intent : Provide support for new parents to properly care for themselves and their children.

Summary : This WELL feature requires paid parental leave for primary and non-primary caregivers and supportive resources for parents returning to work.

Issue : Maintaining an infant's health before birth and during its first months of life is crucial to its long-term health.¹ Though infant care is a universal need, and most countries guarantee a minimum paid maternity leave, many do not offer paid partner leave, and when they do, it is often significantly shorter in duration.² Additionally, shorter and unpaid maternity leave may create barriers to accessing necessary healthcare such as postpartum healthcare, which is critical to receive in the first six weeks after childbirth to support new mothers' health.³

Solutions : The International Labor Organization recommends a minimum of 18 weeks of parental leave, with research indicating that 40 weeks of paid leave results in the greatest overall reduction of risk for low infant birth weight and infant mortality and results in higher rates of on-time infant immunizations.^{1,4,5} Longer paid parental leave and back-to-work coaching programs are associated with numerous health benefits, including higher rates of breastfeeding, reduced infant mortality and postpartum depression, decreased risk of low birthweight infants and higher paternal involvement, as well as higher long-term achievement for children, including reduced school drop-out rates and increased medical appointment attendance.^{4,6-10} Moreover, research shows that parental leave of up to a year (52 weeks) can help improve job continuity for women even years after childbirth.^{11,12} Research also demonstrates that fathers who take paid leave of at least two weeks are far more engaged in childcare nine months after birth.¹³ Organizational support for new parents can also increase employee retention and company loyalty.¹⁴

PART 1 OFFER NEW PARENT LEAVE (MAX : 3 PT)

For All Spaces:

1: Parental leave

A parental leave policy that meets the following requirements is available for all eligible employees:

- At least 40 weeks of parental leave is offered to the designated birthing parent and/or primary care giver.^{4,15} Of this, at least a portion must be paid at 75% or higher of the employee's full salary or wages and include benefits, per the table below.¹⁵
- Parental leave is offered to the non-primary caregiver, of which at least a portion is paid at 75% or higher of the employee's full salary or wages and including benefits, per the table below.^{13,16}
- Leave must be separate from other types of leave (e.g., sick leave, paid time off) and may be used consecutively or non-consecutively during any 12-month period during pregnancy or after birth or adoption.

Tier	Weeks of Paid Leave for the Birthing Parent and/or Primary Caregiver		Weeks of Paid Leave for Non-Primary Caregiver	Point Value
1	At least 12 weeks	AND	At least 2 weeks	1 point
2	At least 18 weeks ^{5,17}	AND	At least 3 weeks	2 points
3	At least 30 weeks ^{4,11}	AND	At least 4 weeks	3 points

2: Parental support policies

The project or organization offers a policy that provides at least two of the following services to help employees utilize and return from parental leave.^{4,6-9}

- At least one of the following upon returning from parental leave:
 - Part-time options (e.g., ramp back programs).
 - Work from home flexibility.
 - Flexible schedules.
- Communications (e.g., emails, modules, trainings) sent to expecting parents about the parental leave policies and resources, including guidance on the positive health impacts of parental leave.
- Coaching or counseling program, or other resources to help employees transition when returning from parental leave.
- Training for managers on how to work with employees to create a plan for parental leave and optimally support employees returning from parental leave.

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C09 NEW MOTHER SUPPORT | O (MAX : 3 PT)

Intent : Provide spaces and policies that encourage and support breastfeeding.

Summary : This WELL feature requires the provision of dedicated lactation rooms with supportive amenities, as well as paid break times, travel accommodations and resources to help mothers initiate and sustain breastfeeding.

Issue : New mothers represent a significant segment of the global labor force, and postpartum care is essential to the health of breastfeeding women and their babies.^{1,2} Research shows that a lack of workplace accommodations contributes to shorter breastfeeding duration or leads to a drop in milk supply, resulting in early weaning.² Nursing parents need a safe, private space with essential amenities to continue breastfeeding or pumping after returning to work.^{2,3} Breastfeeding can help reduce postpartum weight retention and also the risk of breast and ovarian cancer, and is recommended by the World Health Organization, United Nations Children's Fund and American Academy of Pediatrics to support optimal growth and development in infants.⁴⁻⁸ The WHO recommends exclusive breastfeeding of infants up to six months of age, which reduces rates of asthma, allergies and ear infections among children and decreases rates of maternal depression.^{9,10}

Solutions : Supportive breastfeeding programs - such as schedules that provide time for pumping or direct breastfeeding - lactation counseling and travel accommodations can help nursing mothers initiate and sustain breastfeeding.^{2,11} Lactation rooms that promote a comfortable, calm and private space, optimize thermal and acoustic comfort and maximize accessibility needs can support the needs of each individual.¹² Breastfeeding programs can decrease healthcare expenses, reduce employee absences associated with caring for a sick child and increase female employee retention.¹³

PART 1 OFFER WORKPLACE BREASTFEEDING SUPPORT (MAX : 1 PT)

For All Spaces:

1: Breastfeeding support

The project or organization provides a policy to all eligible employees who are breastfeeding that meets the following requirements:

- a. Paid break times for nursing or pumping that last at least 20 minutes at least every 3 hours, with flexibility as necessary to meet individual needs.¹²
- b. One-time coverage or a subsidy of at least 50% for the purchase of a portable breast pump and/or availability of hospital-grade electric pump that accommodates multiple users.¹³
- c. Postpartum lactation counseling, including back-to-work lactation counseling, offered at no cost or subsidized by at least 50%.¹³

2: Support while traveling

The project or organization provides a policy to all eligible employees who are breastfeeding while traveling for business that meets the following requirements:

- a. For all trips: an insulated cooler is supplied at no cost or fully reimbursed.
- b. For all overnight trips: hotels (or other overnight accommodations) with refrigerator access.
- c. For trips lasting longer than 48 hours: full coverage or reimbursement for breastmilk shipping services (e.g., to have expressed milk shipped home).

PART 2 DESIGN LACTATION ROOM (MAX : 2 PT)

For All Spaces:

The project provides at least one dedicated lactation room for all employees that meets the following requirements:

- a. Is at least 4.6 m².¹²
- b. Includes, at a minimum, the following:
 1. Work surface and comfortable chair.¹²
 2. Two electrical outlets.¹²
 3. User-operated lock with occupancy indicator (e.g., signage).¹²
 4. Reservation system, designed to consider privacy preferences (e.g., utilizes a numbering system instead of individual names).^{12,13}
 5. Proximity to sink, faucet, paper towel dispenser and soap. These amenities are not required to be located in a lactation room but may not be located in a bathroom.¹²
 6. Refrigerator with dedicated, sufficient space for milk storage, based on assessment of user needs.¹²
 7. Dedicated microwave or other method to sanitize or sterilize pump equipment, through boiling, steaming, or other sterilizing solutions.¹⁴
 8. Dedicated storage space for pumping supplies.¹²
- c. Provides a calming and comfortable environment that addresses, at a minimum, the following:
 1. Sound minimization.¹²
 2. Ambient lighting.¹²
 3. Thermal comfort.¹²
- d. Present in a quantity that meets current and anticipated demand.¹³

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C10 FAMILY SUPPORT | O (MAX : 3 PT)

Intent : Support working parents and caregivers so that they are able to properly care for members of their family.

Summary : This WELL feature requires projects to offer policies and programs that facilitate childcare and family leave.

Issue : Millions of working individuals take on the role of caregiver for their children, dependents and family members; with the rapidly growing population of older adults, the number of working caregivers will continue to rise.¹ Caregivers often have to adjust their work schedules or take time off, which can significantly impact productivity due to increased absenteeism, workday interruptions and unpaid leave.² An estimated 9% of caregivers left their jobs as a result of having to balance caregiving responsibilities with work obligations.^{2,3} Employers that fail to meet the needs of caregiving employees can see reduced company productivity due to increased absenteeism, workday interruptions and unpaid leave, resulting in an annual average loss of 3.4 billion, 2.8 billion and 1.4 billion dollars, respectively.² Moreover, the loss of a loved one has been associated with higher risks of mortality, physical health problems leading to disability and hospitalization, and psychological stress like insomnia, depression and anxiety.^{4,5} Grief is also tied to productivity loss: in the U.S., grief-inducing experiences cost employers an annual average of \$75 billion in lost productivity and lost business.⁶

Solutions : By offering a range of accommodations, employers can create a supportive culture that meets the diverse needs and caretaking responsibilities of employees.³ On-site childcare that supports flexible schedules can help working parents balance family needs and work demands. Flexible work arrangements can help retain and attract employees while also improving employee productivity and engagement.³ Additionally, financial assistance, support groups and referral to community services can help individuals manage the unique challenges associated with being a caregiver.⁷ Providing employees sufficient time away from work to grieve, in addition to grief counseling and other support, can help reduce employee anxiety, depression and other health risks over time.^{4,8}

PART 1 OFFER CHILDCARE SUPPORT (MAX : 1 PT)

For All Spaces:

At least three of the following are provided for all employees:

- a. On-site childcare centers compliant with local childcare licensure regulations, or subsidies of at least 50% for off-site or at-home childcare.⁹
- b. Back-up childcare coverage (e.g., drop-in daycare, overnight childcare, in-home babysitting service, virtual childcare service) in case of unexpected events (e.g., family emergency, school closure) at no cost or subsidized by at least 50%.¹⁰
- c. School break childcare programs (e.g., center- or home-based care during school break or winter holidays)¹⁰ at no cost or subsidized by at least 50%.
- d. Policy allowing the use of paid sick leave, family leave or personal days for the care of a child.
- e. Policy allowing at least one of the following to support all employees with children:
 1. Part-time options.
 2. Work from home flexibility.
 3. Flexible schedules.⁷

PART 2 OFFER FAMILY LEAVE (MAX : 1 PT)

For All Spaces:

The project or organization makes a family leave policy available to all eligible employees that meets the following requirements:

- a. At least 12 weeks of leave during any 12-month period, paid at 75% or higher of the employee's full salary or wages, for the care of a spouse, domestic partner, child, dependent, parent, parent-in-law, grandparent, grandchild, sibling or other designated relation with a chronic or long-term serious health condition (including an illness, injury, impairment or physical or mental health condition) that involves one of the following:¹¹
 1. Inpatient care in a hospital, hospice or residential healthcare facility for conditions such as stroke, infectious disease or PTSD.
 2. Continuing treatment and/or supervision by a healthcare provider for conditions such as diabetes, asthma or cancer.
- b. The option to use paid sick leave or personal days for the care of a spouse, domestic partner, child, dependent, parent, parent-in-law, grandparent, grandchild or sibling.
- c. At least one of the following for the care of a spouse, domestic partner, child, dependent, parent, parent-in-law, grandparent, grandchild, sibling or other designated relation:
 1. Part-time options.
 2. Work from home flexibility.
 3. Flexible schedules.

PART 3 OFFER BEREAVEMENT SUPPORT (MAX : 1 PT)

For All Spaces:

A bereavement policy is available to all eligible employees that includes the following requirements:

- a. Protocol for notifying supervisors of the loss.

- b. Bereavement leave that includes:
 1. At least five days of paid leave during any 12-month period for the loss of a child (including miscarriages and stillbirths), spouse, parent or dependent.^{12,13}
 2. At least three days of leave, paid at 75% or higher of the employee's full salary or wages, during any 12-month period for the loss of a family member, colleague or friend.^{12,13}
 3. Additional unpaid leave for any of the above losses during any 12-month period, granting employees a minimum of 20 days of leave to use at any point in the bereavement process. The days of paid leave may be counted toward the 20 days.
- c. Bereavement support resources, covering:
 1. Coping with grief.
 2. Returning to work after a loss.^{8,14}
 3. Accessing local bereavement support services.^{8,14,15}
- d. Coverage for bereavement counseling services at no cost or subsidized by at least 50%.

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C11 CIVIC ENGAGEMENT | O (MAX : 2 PT)

Intent : Encourage individuals to become actively involved in and connected to the surrounding community through volunteerism, public spaces and community programming.

Summary : This WELL feature requires a commitment to civic engagement through charitable activities, designated public spaces and community engagement programs.

Issue : Millennials represent an increasingly large segment of the workforce, yet surveys find that one in four millennials plan to quit their current employer in the next year due to the perception that their company's goals do not extend beyond profit.¹ Companies that do not demonstrate stronger social values through civic engagement opportunities, like community volunteering, have been shown to experience lower employee morale, engagement and productivity.² Moreover, research in the U.S. reveals a decline in community social support and relations, with people who feel disconnected from their community encountering more mental health issues than those with a strong community connection.³⁻⁵

Solutions : It is essential for companies to consider the link between corporate social reputation and talent attraction.⁶ Scheduling volunteer opportunities, providing paid volunteer time off, matching employee charitable contributions and working with local community organizations can foster a culture of social responsibility, enhance employee retention and make a positive contribution to the local community.^{1,2,7,8} There are a variety of ways to increase opportunities for civic engagement and establish a culture of social responsibility. Moreover, projects that provide public use spaces and community programming can encourage social interaction and cohesion, community empowerment and collective feelings of ownership, reducing community health risks like stress, depression, heart disease, stroke and chronic disease and improving physical and mental health, happiness and healthy behaviors.^{3,5,9-12}

PART 1 PROMOTE COMMUNITY ENGAGEMENT (MAX : 1 PT)

For All Spaces:

The project or organization has a policy that meets at least two of the following requirements:

- All eligible employees are given the option to take paid time off to participate in volunteer activities for at least the equivalent of two workdays annually (separate from vacation, sick or other generally allocated paid time off).
- A list of local volunteer opportunities is provided to all employees, with at least one suitable opportunity per month and at least eight hours organized by the employer with a registered charity or non-profit per year.
- Employer matches employee's contributions to a registered charity or non-profit of employee's choice, up to a maximum annual amount defined by the employer.
- At least one community engagement program (e.g. events, talks, workshops, trainings or other public engagement intended to promote education, play, physical activity, social connection and/or well-being) at no cost to the public on a quarterly basis on- or off-site.^{3,13,14}

PART 2 PROVIDE COMMUNITY SPACE (MAX : 1 PT)

For All Spaces:

Option 1: Public space

The project designates outdoor or indoor space for public use at no cost that meets the following requirements:

- At least 186 m².¹⁵
- Open at all times, unless closed for security purposes (e.g., during nighttime hours) or temporarily for special events.¹⁵
- Signage or other communication clearly indicates hours the space is open and the designation for public use.¹⁵
- Provides quality seating areas and is easily navigable for individuals of all abilities.^{15,16}

OR

Option 2: Community event space

One or more indoor or outdoor spaces within the project boundary are open for public convenings (e.g., local community groups, student clubs, non-profit organizations) at no cost that meets the following requirements:

- Has the capacity to hold to least 10 people.
- Is available for meetings and/or events (e.g., pop-up health services, community meetings) on a weekly basis, at a minimum.^{3,13,14}

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C12 TALENT RECRUITMENT AND WORKFORCE ACTION PLANS | O (MAX : 3 PT)

Intent : Promote employee engagement and belonging through an assessment of workforce demographics, creation of an action plan to improve engagement and belonging outcomes, implementation of support systems for employees, and implementation of fair hiring and pay practices.

Summary : This WELL feature requires projects to evaluate their current workforce demographics and develop an action plan to address employee engagement and belonging; implement support systems for employees including non-discrimination policies and training, employee resource groups, and dedicated leadership support; and implement fair and transparent hiring and pay practices.

Issue : Global companies in the top quartile for racial and ethnic diversity or gender diversity in executive leadership are 36% or 25% more likely to have financial returns above their national industry medians, respectively.¹ Beyond executive leadership, every 1% rise in ethnic or gender diversity in the U.S. workforce corresponds to a 9% and 3% rise in revenue, respectively.² Despite these performance benefits for organizations, some populations still face barriers to career advancement. In the United Kingdom for example, women's compensation is about about 13% less than men's³ and 35% of LGBTQ+ employees hide that they are LGBTQ+ in feature of discrimination.⁴ In other industrialized countries, 50-70% of persons with disabilities of working age are unemployed.⁵

Solutions : Workforce representation accounts for multiple demographic factors including gender, age, race, ethnicity, socioeconomic status, disability, and more.⁶ Addressing workforce representation is complex and touches upon all of an organization's operations, including hiring practices, determination of compensation and wages, workplace culture and organizational structure. Organizations that seek to hire individuals with different backgrounds and experiences and that espouse fair treatment toward all individuals in their workforce through measurable actions drive both profit and positive culture by attracting and retaining top talent and driving innovation.²

PART 1 CREATE WORKFORCE ASSESSMENT, ENGAGEMENT AND BELONGING PLAN (MAX : 1 PT)

For All Spaces:

The project or organization meets the following requirements:

- a. An assessment of the project or organization's current workforce demographics is conducted utilizing at least four of the following:
 1. Gender (assigned, identity and/or expression).
 2. Sexual orientation.
 3. Race and ethnicity.
 4. Age.
 5. Socioeconomic background.
 6. Disability.
 7. Other metric(s) as identified by the project or organization.
- b. A workforce engagement and belonging plan is established and meets the following requirements:
 1. Identifies strategies to help all employees feel accepted, valued and included, considering findings from the demographic assessment.
 2. Tracks progress toward implementing those strategies on an annual basis.
 3. Publicly summarizes annual progress (e.g., on company website, in annual report).

PART 2 IMPLEMENT WORKFORCE SUPPORT SYSTEMS (MAX : 1 PT)

For All Spaces:

The project or organization meets the following requirements:

- a. Implements a comprehensive non-discrimination policy that includes the following:
 1. Reporting protocol that allows occupants to anonymously report observed or experienced discrimination, resulting in a review by a third party and/or internal human resource professional or other ethics or compliance professional with the offending individual to mitigate future incidents.
 2. Penalties for retaliating against or falsifying reports of bias.
- b. Annual trainings available to all employees that include the following topics:
 1. The benefits having a workforce comprised of people with different backgrounds.
 2. Preventing, identifying and navigating observed or experienced discrimination.
 3. Preventing, identifying and reducing bias.
- c. Provides employees with resources (e.g., budget, meeting space, information, access to external support groups) to establish employee resource groups (ERGs) that are focused on fostering engagement and belonging within the workforce.
- d. Has at least one dedicated senior-level employee whose primary responsibility is to plan and oversee strategies that promote workforce engagement and belonging. The individual must have a dedicated budget and be employed at the executive (C-Suite) level or report directly to a member of the executive (C-Suite) team.¹

PART 3 IMPLEMENT FAIR HIRING AND PAY PRACTICES (MAX : 1 PT)

For All Spaces:

The project or organization meets the following requirements:

- a. Implements a hiring policy that:
 1. Bans the request of salary history.
 2. Requires blind resume reviews (i.e., information is removed that could indicate race/ethnicity, gender and socioeconomic background, including, at minimum, name and home address).
 3. Establishes hiring evaluation protocols with transparent performance standards (e.g., communicating job requirements clearly and establishing fair and consistent performance objectives for all employees, demonstrating transparency and clear expectations for every role).
 4. Establishes goals for hiring candidates with varied backgrounds, perspectives and experiences, which hiring managers are evaluated at performance reviews at least annually.
 5. Establishes having varied backgrounds, perspectives and experiences represented in mid-and executive-level leadership positions and/or on the board of directors.
- b. Implements a compensation policy that is made available to all employees and includes at least three of the following:
 1. Determination of compensation independent of gender identity, sexual orientation, race and ethnicity, age, disability status, or religion.
 2. Provision of compensation that meets basic needs and provides some discretionary income to all employees.
 3. Compensation transparency (e.g., making employee compensation figures visible either internally, externally or both) or published compensation ranges for all titles.⁷⁻⁹
 4. A blind annual evaluation of all employee compensation to assess and improve fair compensation distribution.
 5. Annual trainings or workshops for employees on compensation and contract negotiation.

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C13 ACCESSIBILITY AND UNIVERSAL DESIGN | O (MAX : 4 PT)

Intent : Make buildings more accessible, comfortable and usable for people of all backgrounds and abilities.

Summary : This WELL feature requires projects integrate universal design and wayfinding principles to accommodate diverse needs and create a more inclusive environment.

Issue : More than one billion people, or about 16% of the global population, live with some type of disability, a health condition and/or impairment a person experiences. ²⁰ Many buildings accommodate a variety of individuals with distinct needs and abilities, yet many are not designed with these diverse requirements in mind. ^{5,7} Wayfinding, or knowing how to follow a navigational route, is fundamental for autonomy, social integration and community access for all individuals. ⁴ People who are neurodiverse often lack internal spatial representation and awareness, so they find wayfinding to be particularly challenging and mentally straining. ⁴ The "fear of getting lost" is the most frequent wayfinding challenge for people with cognitive differences. ⁵ As a result, individuals tend to stick to routine or well-known trips to avoid the significant stress associated with navigating new spaces.

Solutions : Universal design addresses multiple aspects of a space, including infrastructure, signage and technologies, and seeks to enhance the opportunity for all individuals to exist independently and comfortably in that space. ¹¹ Inclusive spaces and places should be designed to effectively accommodate a diverse range of individuals. ¹¹ There are several solutions that can enhance universal design for all people and address wayfinding challenges. Easily identifiable landmarks, such as sculptures, artwork and distinct architectural elements, can serve as significant points of reference during the navigational process as they are often memorable. ³² Additionally, printed and digital signage provides important information for navigation as well as at points for decision-making. ² Simple and user-friendly sitemaps that are strategically positioned at key locations throughout a building, including entrances, can quickly help individuals orient and move throughout a space. ¹¹ Moreover, electronic maps can allow individuals to preview routes before engaging in travel. ^{7,22} This format is especially helpful for people who are neurodivergent as it offers a more equitable solution to wayfinding and provides individuals with greater control and the ability to choose how they navigate the environment. ²² Lastly, providing wayfinding information in various formats that support a diverse range of cultures and abilities (e.g., braille, different languages) enhances access, allowing all individuals to more easily navigate spaces. ¹¹

PART 1 INTEGRATE UNIVERSAL DESIGN (MAX : 2 PT)

For All Spaces:

The project considers best practices in universal design to accommodate a diverse range of disabilities and needs by implementing at a minimum one strategy in each of the following categories: ⁹

- a. Physical access: entry, exit and key interaction points that enable inclusive entrance to the project and strategies that accommodate user changes as needed (e.g., stair-free entrances, step-free egress, operable windows, automatic doors), supporting ease and independence of use. ^{1,7,10}
- b. Developmental and intellectual health, including sensory requirements of people who are neurodiverse: strategies that use color, texture, images and other multi-sensory, visually perceptible information. ^{1,10,11}
- c. Wayfinding: strategies that help individuals intuitively navigate through the project (e.g., signage, tactile maps, symbols, auditory cues, information systems, images, color that considers color blindness, various languages). ¹⁰
- d. Policies and programs: strategies that support inclusion and accommodate a diverse range of needs (e.g., diversity and inclusion training, flexible work hours for individuals with disabilities). ^{1,7,10}
- e. Technology: technology (e.g., audio and visual equipment, web access, QR codes) that helps individuals fully utilize a space (e.g., remote access to assist blind or deaf individuals, support for those who do not speak the native language), made available to all occupants at no cost. ^{1,7,10}
- f. Safety: strategies that support easy access to all spaces and amenities and minimize risk of injury, confusion or discomfort (e.g., lighting or clear sightlines to increase feelings of security, service animals, emergency egress plans with highlighted exit points). ^{1,10,11}

PART 2 B SUPPORT INCLUSIVE INTERIOR NAVIGATION (MAX : 1 PT)

For All Spaces:

Primary circulation routes meet the following requirements:

- a. Include landmarks (e.g., sculptures, water features, artwork, architectural elements) that are:
 1. Positioned at points-of-decision (e.g., intersections, elevators, stairways and emergency exits). ^{21, 22}
 2. Physically prominent and unique (e.g., due to shape, color, size, historical or cultural significance). ²¹
- b. Include amenities (e.g., restrooms, quiet areas, drinking water stations, information services) that are:
 1. Within a clear line of sight to another landmark or amenity.
 2. Located consistently throughout the project boundary (e.g., same location on each floor). ²¹
- c. Include seating areas that are located consistently throughout the project boundary (e.g., same location on each floor). ²¹

Note :

This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

PART 3 B SUPPORT INCLUSIVE BUILDING & NEIGHBORHOOD WAYFINDING (MAX : 1 PT)

For All Spaces:

Information is provided for the building in a way that meets the following requirements:

- a. Is present in a physical format (e.g., printed signage, digital display) at the main building entrance and at least one additional functional building entrance, if present.
- b. Is available in an electronic format (e.g., website, mobile application).
- c. All formats meet the following:
 1. Include audio and/or braille that is compliant with ICC/ANSI A117.1-2003.
 2. Use clear and simple language.⁹
 3. Include more than one language.⁹
 4. Include contact information for assistance services (e.g., screen readers, listening systems, wayfinding apps).
- d. All formats include a local amenities directory that meets the following:
 1. Displays the location of at least 8 existing use types (as defined in Appendix V1) relative to the building.
 2. Displays the location of all public transit stops located within a 400 m [0.25 mi] walk distance of the main building entrance, if present.
- e. If multiple businesses are present, all formats include a business directory that shows the names of businesses located within the building and their locations (e.g., floor level, room number).
- f. All formats include a site map of the building that shows the following, if present, using easily recognizable symbols:
 1. Bathrooms.
 2. Drinking water stations.
 3. Information services.
 4. Areas for resting or sitting.
 5. Wayfinding landmarks.²¹
 6. Accessible pathways.²²
 7. Emergency exits.
- g. All formats include a sensory map of the building that shows the following, if present, using easily recognizable symbols.^{9,22}
 1. Restorative spaces.
 2. Loud sounds.
 3. Crowded spaces.
 4. Flashing lights.
 5. Strong smells.

Note :

This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

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C14 EMERGENCY RESOURCES | O (MAX : 2 PT)

Intent : Provide resources, personnel and training to help organizations, families and individuals respond to diverse emergency situations.

Summary : This WELL feature requires projects to offer resources like first aid kits, automated external defibrillators (AEDs) and opioid response kits, coordinate with emergency response teams and provide emergency preparedness and response trainings.

Issue : It is estimated that sudden cardiac arrest (SCA) causes between 6.8 – 8.5 million deaths worldwide per year, with a global survival rate of less than 1%; in the United States, about 10,000 SCA deaths per year occur in the workplace.^{1,2} An SCA victim's chances of survival lower by 7-10% with every minute that passes without cardiopulmonary resuscitation (CPR) or defibrillation.³ Additionally, nearly 16,000 people worldwide die from preventable injuries each day, yet in most European countries, only 5-10% of the population is trained in first aid.^{4,5} While natural disasters kill an average of 90,000 people annually, nearly 60% of American adults have not practiced what to do in a disaster.^{6,7} Moreover, anaphylaxis causes up to 1,500 deaths per year in the U.S., with studies showing a delay in administering epinephrine to be a significant risk factor associated with fatal outcomes from allergen exposure.⁸⁻¹⁰ Finally, drug overdose is the leading cause of accidental death in the U.S., with overdose from opioids driving the epidemic.¹¹

Solutions : Rapid and effective emergency response requires coordination with local emergency responders and maintenance of emergency resources such as an emergency notification system, first aid kits and AEDs.^{12,13} Supplementing those resources with occupant training on CPR, first aid, AED use and individual and family preparedness can increase individual response time and help improve survival rates; CPR and AED training alone can increase victim survival rates by nearly 40%.^{3,12} In food allergy emergencies, quick access to and immediate availability of epinephrine is essential.^{10,14} Finally, increasing the availability of naloxone is a critical component of reducing opioid-related overdose deaths, with evidence suggesting that when naloxone and overdose education are available to community members, overdose deaths decrease in those communities.¹⁵

PART 1 PROMOTE EMERGENCY RESOURCES (MAX : 1 PT)

For All Spaces:

1: Emergency resources

Resources are in place that support emergency response, including at least three of the following:

- a. Information indicating emergency procedures (e.g., evacuation during fire or earthquake, containment and response strategies for infectious disease outbreaks, shelter-in-place during active shooter) available to all guests upon entrance to the building.
- b. Building emergency notification system with auditory and visual indicators of emergency (e.g., public address systems, flashing lights).
- c. At least one first aid kit per floor meeting requirements of Appendix C3.
- d. AEDs accessible to any occupant within 100 meters and adoption of routine maintenance and testing schedule.^{16,17,18} The locations of building AEDs are identified through posters, signs or other forms of communication other than on the AED itself.¹⁷
- e. Undesignated epinephrine auto-injectors for food allergy emergencies.¹⁴
- f. Rides for employees subsidized or reimbursed by at least 50% to destination of need for emergency situations (e.g., urgent medical needs, personal or family emergency), including from home to work as needed (e.g., during public transit shutdown).

2: Emergency training and personnel

At least two of the following are in place:

- a. Emergency response team for medical emergencies, including at least one certified medical professional, first responder or other qualified personnel who has received emergency medical training (e.g., Emergency Medical Technician, paramedic, police, fire service, individuals certified in advanced first aid) present within the building during regular business hours.^{17,18}
- b. Security or crisis response team for human-caused disruptions (e.g., active shooter, civil unrest).
- c. Annual availability to regular occupants of a certified training course on CPR, first aid and AED usage.¹⁷
- d. Trainings to promote emergency preparedness available to regular occupants that address at least the following topics:
 1. Creating evacuation or sheltering plans.
 2. Building emergency kits, supplies and go-bags.
 3. For residents, if applicable, planning communications with family or primary contacts in case of emergency.

PART 2 PROVIDE OPIOID RESPONSE KIT AND TRAINING (MAX : 1 PT)

For All Spaces:

1: Opioid response kits

The following requirements are met:

- a. All emergency preparedness or first aid kits include:
 1. Naloxone rescue kits. Projects may choose a single dose nasal spray, a multi-step nasal spray, a single step injection or a multi-step injection.
 2. Instructions for how to prepare and administer naloxone, as well as immediate next steps after administration.
 3. A list of who on-site has received opioid response training, and their contact information.

- b. Protocol is in place for follow-up after an opioid emergency event, including a plan for:
 1. Debriefing those affected.
 2. Immediate replacement of naloxone kit following use.
 3. Replacing expired kits.

2: Opioid response training

The following requirement is met:

- a. Regular occupants receive opioid emergency training in-person or virtually, covering:
 1. General information about opioid use and naloxone.
 2. Recognizing the signs of an overdose and immediate steps to take.
 3. How to safely administer naloxone and steps to take following administration.

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C15 B EMERGENCY RESILIENCE AND RECOVERY | O (MAX : 4 PT)

Intent : Better enable individuals and communities to maintain health and well-being, and organizations to maintain business function, during and after emergencies.

Summary : This WELL feature requires projects to create a business continuity plan, establish a re-entry plan and offer supportive resources to facilitate resilience during, and recovery after, an emergency.

Issue : Today, communities must prepare for and face emergencies ranging from natural disasters, civil unrest to public health crises. Globally, the frequency, scale and economic and social cost of disasters is increasing due to climate change, population growth and rapid urbanization.¹⁻³ In 2019, the global economic losses from disasters amounted to \$232 billion, and the 2020 COVID-19 pandemic caused the largest global recession in history.^{1,4} Small businesses may be particularly vulnerable as research shows that about 90% of smaller companies fail after emergencies unless they can resume business operations within five days.⁵ Emergencies such as biological events or active shooters may necessitate sheltering in the workplace, while longer-term emergencies may lead to extended workplace shutdowns.⁶⁻¹³ The latter can result in extensive layoffs, while employees who continue working may be forced to work in higher-risk conditions or remotely in spaces not equipped to support productivity.⁹⁻¹³

Solutions : Business continuity planning is critical to help manage business disruption, restore business operations, minimize risk to employees and mitigate financial loss when emergencies occur.¹⁴⁻¹⁷ Establishing organizational remote work readiness can help operations run smoothly and support employee well-being and productivity when an emergency makes remote work imperative.^{14,18} Moreover, employer-funded employee relief assistance can support both employee retention and well-being during and after emergencies.¹⁹⁻²¹ For emergencies that require sheltering on-site, a shelter-in-place plan is crucial to supporting occupant safety.^{6,8,22} Designating space for emergency public use can reduce the burden on medical facilities and help patients receive immediate care.^{23,24} Re-entry plans after emergencies should consider employee needs, offer employees flexibility, assess critical infrastructure systems, communicate re-entry strategies to key stakeholders and re-evaluate existing policies, operations and protocols to support a healthy, safe and inclusive re-entry.²⁵⁻³⁴ In certain types of emergencies, including epidemics and pandemics, re-entry plans may need to consider relevant vaccination, testing, and other requirements (as applicable), including social distancing and use of personal protective equipment in order to help manage risks.³⁵

PART 1 PROMOTE BUSINESS CONTINUITY (MAX : 1 PT)

For All Spaces:

Projects implement a business continuity plan (BCP) that addresses at minimum the following:^{14,18}

- a. Determines critical business functions, processes, supporting resources and dependencies (e.g., email, internet connectivity, third-party suppliers or service providers, interdependent departments).
- b. Includes a list of the roles and responsibilities of the business continuity team and convenes the team annually (at minimum) to review, test and update (as needed) the plan.
- c. Implements a business impact analysis to evaluate the likely effects resulting from disruption of normal business functioning due to a disaster and to identify which critical business functions should be prioritized for recovery.
- d. Conducts a remote work readiness assessment, including at minimum the following:
 1. Evaluates which employees and/or positions (if any) are able to work remotely.
 2. Evaluates which employees and/or positions (if any) have the necessary support infrastructure to work productively in a remote situation.
 3. Evaluates whether organizational technology (e.g., company laptops, virtual private network (VPN)) is set up to support enterprise-wide remote work.
 4. Implements the strategies necessary to support remote work readiness as determined by the evaluation, including (as applicable) methods of communication to employees during remote work and provision for alternate work locations.
- e. Outlines strategies to support short- and long-term continuity in various disasters (e.g., blizzard, pandemic), restore and maintain business operations following disruption and re-mobilize in response to recurring disasters.

Note : This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

PART 2 SUPPORT EMERGENCY RESILIENCE (MAX : 1 PT)

For All Spaces:

Projects implement at least one of the following:

- a. Designated outdoor or indoor space is made available to emergency responders, relief organizations or other equivalent institutions at no cost for alternative use in case of emergency (e.g., shelter during a natural disaster, treatment area during a pandemic).
- b. Funding or other resources (e.g., in partnership with local agencies providing relevant services or resources such as vouchers, shelter, clothing, food, transportation) are provided by the employer for emergency use by employees in at least two of the following critical scenarios:
 1. Sheltering from domestic violence or abuse.
 2. Quarantine due to infectious disease exposure.
 3. Damage to employee housing from a disaster.
- c. Shelter-in-place plan for emergencies in which occupants cannot leave the project (e.g., hurricane, chemical spill) that includes the following:
 1. A shelter-in-place kit with resources to help occupants shelter in place within the project for at least 24 hours (e.g., water, food supplies, blankets, flashlights, first aid kit).⁸

2. A process for occupants or groups who may be more vulnerable (e.g., older adults, people with disabilities, pregnant women, children) to confidentially identify specific needs they may have during a shelter-in-place emergency.²²
3. Procedures for communicating to occupants the decision to evacuate or shelter-in-place during an emergency.
4. A commitment to incorporate shelter-in-place guidelines provided by a relevant local-, regional- or global-level emergency response agency (e.g., WHO, government emergency management agency or equivalent) into the plan, and to adhere to instructions provided by that agency during a shelter-in-place emergency.
5. Annual (at minimum) occupant trainings on the shelter-in-place plan.

Note : This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

PART 3 FACILITATE HEALTHY RE-ENTRY (MAX : 1 PT)

For All Spaces:

Projects establish a plan for re-entry into the project after an emergency (e.g., natural disaster, public health emergency) addressing at minimum the following:

- a. Consultation with regular occupants before and after re-entry to understand their needs and concerns related to re-entry.
- b. Safety, compliance and risk inspections of water, mechanical, electrical, ventilation and life safety systems, including necessary actions to restart building and facility systems after prolonged shutdown and approval or clearance for safe re-entry, as applicable.
- c. A list of roles for those who will be responsible for overseeing the re-entry plan. While roles and contact information should be made available to an organization's personnel, it is not necessary to include this information in the plan submitted for purposes of verifying this feature.
- d. Re-evaluation and adjustment (as needed) of human resources, workplace wellness and employee support policies and amenities (e.g., use of common areas and shared spaces like wellness rooms, food provision, physical activity programs) to support a safer and healthier re-entry.
- e. Policy to support phased re-entry (as needed) offering part-time options, work from home flexibility and/or flexible schedules for all employees (as feasible), particularly for parents and caregivers who may have specific dependencies (e.g., due to childcare closures or a sick family member) and vulnerable groups (e.g., people with disabilities or who may be particularly vulnerable to infectious disease).
- f. Re-evaluation and adjustment (as needed) of facilities management policies and organizational protocols to support safer and healthier re-entry, including but not limited to:
 1. Crowd management and spacing and physical distancing of individuals.
 2. Heightened security measures (e.g., screening, security personnel).
 3. Access to personal protective equipment (PPE).
 4. Additional sanitation supplies and other cleaning or maintenance protocols.
- g. Contingency planning and re-closure measures should the same hazard that forced initial closure re-occur.
- h. Frequent communications through multiple methods (e.g., emails, signage, trainings) to all relevant stakeholders, including (as applicable) employees, occupants, residents, facilities management team, contractors and community members, on the re-entry plan, new or altered policies, operations and procedures, relevant local-, state-, national- or global-level re-entry guidelines and how the project will address occupant health and safety concerns.
- i. Evaluation and incorporation of re-entry guidelines (as available) provided by a relevant local-, regional- or global-level emergency response agency (e.g., WHO, government emergency management agency or equivalent) into the plan, and adherence to instructions provided by that agency during re-entry.

Note : This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

PART 4 ESTABLISH HEALTH ENTRY REQUIREMENTS (MAX : 1 PT)

For All Spaces except Dwelling Units:

Projects located in a region with heightened risk of infectious respiratory disease transmission defined by a public health authority (e.g., World Health Organization, local public health agency) require at least one of the following for regular occupants to enter the space:

- a. Proof of vaccination or documented exemption (e.g., a medical or religious exemption), including necessary

boosters as applicable, for the disease of heightened risk.

b. Both of the following:

1. Proof of negative diagnostic testing for the disease of heightened risk and, for eligible employees (as applicable), access to diagnostic testing at no cost.
2. Face masks worn indoors by, at minimum, unvaccinated occupants. Face masks are available for all occupants.³⁶

Note : This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

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C16 B AFFORDABLE HOUSING | O (MAX : 2 PT)

Intent : Promote housing equity through the allocation of healthy affordable housing units.

Summary : This WELL feature requires projects to designate affordable housing units that are tenure blind, reduce housing costs for low-income tenants and offer multi-bedroom options.

Issue : Families unable to find affordable housing spend a significant share of their income on housing costs, leaving insufficient resources to cover other basic needs such as food, clothing, utilities and medical care.¹⁻³ Across the United States, European Union, Japan and Australia, over 60 million households are financially stretched by housing costs.⁴ With lack of affordable housing access, an estimated 235 million urban families live in substandard housing, which exposes them to mold, dust, water leaks, lead-based paint, poor air quality, temperature extremes and vermin, leading to poor health outcomes like asthma, infectious disease, cardiovascular events and children's nervous system damage.^{1-3,5,6} In 2016, only 3.2 million affordable housing units were available for the 10.4 million extremely low-income households in the United States; in India, there is a deficit of 11 million affordable units.^{7,8} In rural England, only 8% of housing stock is affordable compared to 20% in urban areas.⁹ Affordable housing shortages lead to homelessness, which increases stress, substance use and morbidity in adults and mental health issues and depression in youth.^{3,6,10}

Solutions : Increasing housing affordability, quality and safety improves resident health, feelings of security and self-esteem, and increases developmental ability and nutrition levels in children.^{3,6} Affordable housing access can help prevent communicable diseases, improve overall health and provide a stable and efficient platform for the delivery of food, healthcare and essential services, especially for vulnerable groups such as the elderly, children, and individuals with chronic illnesses or disabilities.^{1,3,5,6,10} With rising urbanization, healthy affordable housing will be critical for community health promotion.

PART 1 ALLOCATE AFFORDABLE HOUSING (MAX : 2 PT)

For Dwelling Units:

The project meets the following requirements:

- a. A percentage of dwelling units are allocated for tenants whose incomes are at or below an income limit relative to the local median household income [e.g., Area Median Income (AMI)] and adjusted for family size. Recognition is awarded as per the selected tier in the table below:

Tier	Units Allocated and Income Limited	Point Value
1	20% of units or more, 0 - 50% of local median OR 40% of units or more, 51 - 80% of local median	1 point
2	100% of units, 0 - 80% of local median	2 points

- b. Total annual housing costs (i.e., rent and utilities) paid by affordable unit tenants are less than 30% of the income limit selected in requirement (a).
- c. Housing costs are maintained at the levels described in this feature for the duration of a project's engagement with WELL.
- d. Affordable housing units are not visually or functionally distinctive from market-rate units (if present) and have the same access point into the building.
- e. In projects with 10 or more affordable housing units, at least 50% of allocated units have two or more bedrooms and at least 10% of allocated units have three or more bedrooms.

Note : This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

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C17 B RESPONSIBLE LABOR PRACTICES | O (MAX : 3 PT)

Intent : Promote organizational commitment to responsible labor practices in order to address modern slavery in the supply chain and support human rights.

Summary : This WELL feature requires projects to evaluate and disclose unfair labor practices associated with modern slavery in their operations and supply chain, specifically in the areas of construction, cleaning and catering, and to take steps to address modern slavery in the supply chain.

Issue : Modern slavery refers to the various situations in which a person is recruited, transported or compelled to work through force, fraud or coercion.^{1,2} Modern slavery practices may include traditional slavery (or involuntary servitude), human trafficking, forced labor, bonded labor, sex trafficking and the worst forms of child labor.^{1,2} The Global Slavery Index estimates that in 2016 there were over 40 million victims of modern slavery worldwide, including 24.9 million in forced labor.³ Modern slavery is recognized as a violation of human rights, including a right to health, and as a global public health issue.⁴⁻⁷ Research shows modern slavery has severe consequences for victim health and well-being, including high risk of physical injury, mental health issues like anxiety, depression and post-traumatic stress disorder (PTSD), exposure to infectious disease, suicide and limited access to healthcare.⁴⁻⁷ Reports estimate that a majority of modern slavery victims – nearly 30 million – reside in the Asia Pacific region, where many global supply chains lead.⁸⁻¹⁰ Of the 195 countries in the world, only 40 have investigated labor exploitation in supply chains and almost half of all countries worldwide have yet to criminalize slavery.^{11,12} Industries found to be at highest risk for modern slavery include clothing, electronics and technology manufacturing, food and agriculture (including catering), and construction.^{13,14} The cleaning sector is also considered high-risk given the complexity and opaque operations of the industry; in 2011, the Fair Work Ombudsman (FWO) reported that out of 315 Australian cleaning companies analysed, 37.1% were non-compliant with responsible labor practices, including underpayment of wages and lack of recordkeeping.^{14,15} Businesses often do not sufficiently assess modern slavery risks in their supply chains due to the complexity of supply chains reaching across sectors and regions; for example, in 2016 only 47% of UK manufacturers had adequately identified all of the suppliers in their supply chain.^{13,14}

Solutions : Businesses can play a critical role in helping to identify and prevent occurrences of modern slavery.¹⁶ To address modern slavery in the supply chain, companies must first establish due diligence processes that comprehensively detect risks and influencing conditions.¹⁴ The UK Modern Slavery Act of 2015 requires companies to establish and disclose a risk assessment process, anti-slavery policies and steps taken to address identified risks.¹⁷ The Australian Modern Slavery Act of 2018 similarly requires entities to publish an annual report on actions they have taken to address modern slavery in their operations and supply chains.¹⁸ Best practices also include ethical procurement policies, employee training on relevant policies and risk identification as well as engagement with Tier 1 suppliers and established processes for incident reporting, while “leadership” approaches include standalone modern slavery or human rights policies, deeper risk assessments into Tier 2 suppliers through Tier 6 suppliers, capability-building with suppliers, strong remediation mechanisms and slavery-specific performance metrics.^{13,19} Implementing these steps also supports the achievement of U.N. Sustainable Development Goal 8: Decent Work and Economic Growth, which calls for eradicating forced labor, modern slavery, human trafficking and child labor by 2025.²⁰

PART 1 DISCLOSE LABOR PRACTICES (MAX : 1 PT)

For All Spaces:

The project or organization meets the following requirements:

- a. A comprehensive mapping of the project’s or organization’s structure, operations and supply chains is conducted annually for Tier 1 suppliers in the following sectors (as applicable):²¹
 1. Construction.
 2. Cleaning.
 3. Catering.
 4. Security.
 5. Maintenance.
- b. A risk assessment is conducted annually that evaluates risks in the project’s or organization’s operations and Tier 1 suppliers (at a minimum) in the above sectors for the following practices associated with modern slavery:^{21,22}
 1. Worst forms of child labor.
 2. Forced labor.
 3. Traditional slavery.
 4. Bonded labor.
 5. Human trafficking.
- c. A report is completed annually that discloses the following information, is reviewed by executives in the C-Suite, board of directors and/or equivalent high-level stakeholders, and published on the project or organization’s website:²²
 1. Processes of evaluation and risk assessment.
 2. Results of evaluation or risk assessment, including where modern slavery risks have been identified.
 3. Statement of commitment (including established goals and policies) aimed at identifying, preventing and mitigating modern slavery practices in the project’s or organization’s operations and supply chain.

Note : This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

PART 2 IMPLEMENT RESPONSIBLE LABOR PRACTICES (MAX : 2 PT)

Note : Projects may only achieve this part if Part 1 is also achieved.

For All Spaces:

The project or organization implements an action plan that meets the following requirements:^{21,22}

- a. Establishes annual targets for the prevention and/or mitigation of modern slavery in their operations and supply chain in the following areas (as applicable):
 1. Construction.

2. Cleaning.
 3. Catering.
 4. Security.
 5. Maintenance.
- b. Describes how the effectiveness of the implemented strategies are assessed annually and updates targets and/or strategies accordingly.²¹
- c. Addresses implementation of the following strategies to meet established targets:^{21,22}
1. Anti-slavery and anti-human trafficking policies.
 2. Responsible procurement policies.
 3. Annual trainings, mandatory for employees involved in procurement and made available to all employees, educating about the consequences of modern slavery and the project or organization's policies and steps for preventing, identifying and reporting observed or potential incidences of modern slavery practices.
 4. Reporting protocol that allows employees and supply chain Tier 1 suppliers to anonymously report modern slavery risks and practices.
 5. Process for review and remediation of any identified modern slavery practices to prevent and mitigate future incidents.
 6. Process for consultation and revision of contracts, including establishing supplier obligations to address modern slavery, with any suppliers that have been identified as high risk for modern slavery practices.
- d. The requirements in a, b and c are met for supply chain Tiers according to the table below.

Feature Tier	Supplier Level	Point Value
1	Supply Chain Tier 1	1 point
2	At least Supply Chain Tiers 1 & 2	2 points

Note : This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

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C18 B SUPPORT FOR VICTIMS OF DOMESTIC VIOLENCE | O (MAX : 2 PT)

Intent : Increase availability and access to support services, resources and care for victims of domestic violence.

Summary : This WELL feature requires projects to implement a policy that supports victims of domestic violence and to educate employees on the domestic violence resources that are made available by the project.

Issue : Domestic violence is the most common form of gender-based violence across the world.¹ Women are disproportionately affected, with violence most commonly perpetrated by men against women.² The World Health Organization estimates that around 30% of women who have been in a relationship experience some form of physical and/or sexual violence perpetrated by an intimate partner.³ The impact of this violence is severe: globally, it is estimated that up to 38% of murders of women are committed by a male intimate partner.⁴ Beyond fatal outcomes, domestic violence can lead to a range of adverse health consequences, including higher risk of injuries, sexually transmitted infections (including HIV), depression, anxiety and substance misuse.¹ Moreover, the social and economic consequences of domestic violence are significant, including loss of wages and inability to work, as well as increased risk of homelessness.^{1,3,4} The deep impacts of domestic violence are not just felt at home, but also carried into the workplace: research shows that at work, victims are more likely to have reduced performance and productivity and increased absenteeism and turnover.^{5,6}

Solutions : Workplaces can play a role in responding to domestic violence through policies designed to protect and support victims.⁵⁻⁸ It is also important for workplaces to develop supportive and non-judgemental environments in which employees feel comfortable and safe disclosing any violent situation they may be facing.⁷ Employers can help protect victims through various measures, such as confidential protocols for reporting, reviewing and responding to an incident, ability to change workplace location and start and finish times, increased security for workplace access, call screening procedures and specific emergency response procedures.^{5,7,8} Countries such as New Zealand, the Philippines and parts of Canada have introduced legislation that promotes best practices among employers to support victims of domestic violence, including paid time off for victims and financial resources to support relocation.⁹⁻¹¹ By providing policies and resources to support victims and educate employees, employers may play a role in helping to reduce the physical and mental impacts of domestic violence.^{7,12}

PART 1 SUPPORT VICTIMS OF DOMESTIC VIOLENCE (MAX : 2 PT)

For All Spaces:

1: Domestic violence policy

The project or organization makes available to all eligible employees a domestic violence policy that meets the following requirements:

- a. Provides employees who are victims of domestic violence at least ten days of leave, paid at the employee's full salary or wages, during any 12-month period. Leave must meet the following requirements:^{5,7,8,12}
 1. Distinct from paid time off, sick leave and family leave.
 2. If requiring incident disclosure for employees to qualify, takes steps to protect employee privacy.
 3. Does not require a prerequisite minimum qualifying period of employment before an employee is eligible to take leave.
- b. Outlines a clear protocol for incident reporting and response that includes the following:
 1. Process for employees to confidentially report incidents of domestic violence, including one or more designated contacts that employees can approach confidentially for support when reporting incidents.⁸
 2. Process of incident response that includes consultation with the victim, prioritizes victim privacy and safety and ensures incident disclosure will not adversely impact victim employment status.
- c. Offers employees who report domestic violence incidents at least two of the following:
 1. Flexible working arrangements (e.g., adjusted work hours or location).^{5,7,8,12}
 2. Heightened security measures (e.g., call screenings, controlled workplace access, duress alarms, changes to contact information, worksite security escorts).^{2,5,8}
 3. Referrals to local support organizations, community groups and crisis lines, including those available through Employee Assistance Programs (EAPs).^{8,12}
 4. Temporary accommodations or financial support to cover the costs of temporary accommodations.⁵
- d. Policy and related resources provided by the organization are easily and confidentially available (e.g., via a health portal, mailed communications, employee website) to all employees and reviewed and adjusted (as needed) annually by the organization. Policy must be made available to all new employees during onboarding.^{2,5}

2: Employee education

The project or organization offers in-person or virtual trainings (e.g., workshops, seminars) that meet the following requirements:

- a. Are required of all managers and made available to all employees.^{2,8}
- b. Covers the following topics:^{7,12}
 1. The relevant domestic violence policy and resources.
 2. Signs and symptoms that a person may be a victim of domestic violence.
 3. How to appropriately respond if a colleague or direct report discloses that they or another employee is experiencing domestic violence.

Note :

This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the

world.

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C19 B EDUCATION AND SUPPORT | O (MAX : 2 PT)

Intent : Support career growth by reducing educational and professional advancement barriers.

Summary : This WELL feature requires financial support for education and/or mentoring opportunities to support career advancement and financial stability.

Issue : Creating supportive programs for education, mentorship and sponsorship can positively impact employee financial health and opportunities.¹ Such support can have short and long-term effects on many dimensions of an employee's life and can benefit other members of their immediate family including spouses, children and parents.² Systemic discrimination and the resulting disparities in access to economic, social and educational support have historically kept marginalized populations from achieving gainful employment.^{3,4} Financial wellness also impacts mental health, housing stability and food security.⁵

Solutions : Studies have shown that minorities and women significantly benefit from peer-to-peer mentors and sponsors who champion them.^{4,6,7} Sponsors provide a deeper level of mentorship to their proteges.⁸ Through exposure and relationships, they provide career advancement opportunities for the protegee when they are "in and out of the room".⁸ Mentorship develops trust between the two parties, creating a foundation for a successful relationship between mentor and mentee.⁴ Mentorship surveys also reveal satisfaction amongst employees within such programs – in the United States, 71% of employees with a mentor say their company provides them with excellent or good opportunities to advance their careers.⁷ Employees may also benefit from other support options such as speaker sessions, workshops and seminars.⁹ Guest speakers can help advance professional development, spark new ideas and offer follow-up training opportunities.¹⁰ Providing subsidized financial assistance to promote education for marginalized populations may create more opportunities for career advancement and, therefore, bring greater financial stability for individuals and their families.¹¹⁻¹³ Research from the United States found that 80% of employees agreed that their employer's tuition assistance program makes them more likely to stay with the organization, and 71% of respondents rated tuition assistance as the best or among the best benefits offered by their employer (excluding health care benefits).^{1,14} Financial assistance for education and peer-to-peer mentorship positively impacts all employees, and in particular, marginalized populations.^{7,13}

PART 1 ESTABLISH EDUCATION AND SUPPORT (MAX : 2 PT)

For All Spaces:

Option 1: Tuition assistance

The project or organization provides a Tuition Assistance Program (TAP), which may be limited to select institutions, to all eligible employees that meets the following requirement:

- a. Pays for a minimum of 75% of educational expenses for all enrolled courses each term/year (including tuition, program fees and books/materials) for vocational training, undergraduate, graduate, certificate courses and similar educational goals. Assistance is structured per the table below:

Tier	Tuition Assistance Structure	Point Value
1	Reimbursements for educational expenses	1 point
2	Direct payments for educational expenses	2 point

OR

Option 2: Mentorship program

The project or organization provides a mentorship or sponsorship program that meets the following requirements:

- a. A process for matching mentor to mentee and/or sponsor to protégé (e.g., interest form).
- b. A plan development process co-created between mentor and mentee or sponsor and protégé to identify the needs, goals and objectives. The plan should be customized and focus on individual strengths, personality, skills and workstyles.
- c. A process by which mentees and protégés report on meetings between the mentor/mentee or sponsor/protégé and participation in activities recommended by the mentor or sponsor.
- d. An allocated budget for specialized resources or training related to professional development (e.g., conferences, courses, assessments, workshops, group sessions) for all participating mentees and/or protégés.^{15,16}
- e. Mandatory training about explicit and implicit bias (in the form of educational seminars, workshops, classes or on-demand modules) for all participating employees before engagement in the program.

Note : This Option is worth 1 point

Note :

This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

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C20 B HISTORICAL ACKNOWLEDGEMENT | O (MAX : 1 PT)

Intent : To acknowledge and honor historical rights, ownership and occupation of land and/or recognize significant societal contributions of marginalized communities.

Summary : This WELL feature requires projects to develop a comprehensive program that acknowledges the history of colonization, displacement and relocation and/or the significant contributions that Indigenous, enslaved and migrant peoples have made to a particular place.

Issue : Specific ethnic and racial populations have historically been colonized, enslaved, marginalized, abused and neglected (for the purposes of this feature, collectively referred to as "marginalized" communities), with the threat of erasure due to imperial power dynamics, laws and decrees across the globe. These communities were the original stewards of land and biodiversity for millennia, making significant contributions to society through traditional and ancient knowledge.¹⁻³ On a global scale, Indigenous Peoples and people across the African diaspora have witnessed their rightful and sacred lands being taken against their will through Doctrine of Discovery, broken agreements, and abuse of eminent domain and legal constructs such as heirs' property laws.⁴⁻⁶ In the United States, communities of color, such as Asian and African Americans, have made significant contributions to the building of the country through forced and unjust labor.^{2,3} In addition, significant contributions through forced labor have occurred in South America and the Caribbean region.⁶⁻⁹ The harmful impacts of global colonization have led to disparities in physical, mental, emotional and economic well-being, long-term displacement, erased histories, and a breakdown of community, social and economic structures, the effects of which have passed down through generations.¹⁰⁻¹² Studies have shown this has caused historical trauma and that the mental health of Indigenous youth still suffers on the Historical Loss Scale.¹³ This intergenerational trauma has been found in DNA at the cellular level.¹⁴ In Australia, New Zealand and North America, recent truth and reconciliation acknowledgments, laws and reparations have begun for Indigenous People who have suffered various human rights violations, including land rights abuse and attempted genocide.^{4,12,15} It is important that organizations work with representatives from within the affected community and/or Tribal Nation to ensure that their acknowledgement and reconciliation and/or reparations programs do no harm, as defined by the community.

Solutions : This feature seeks to acknowledge and begin reconciliation for at least a portion of the valuable contributions from affected populations by encouraging healing and connection that is grounded in place and culturally-relevant values.¹⁶⁻²⁰ The act of reconciliation illuminates and drives awareness of historical contributions of marginalized and colonized communities through action and future-facing programs (i.e., land acknowledgments, truth and reconciliation practices, empowerment programs, financial relief and economic development).

PART 1 PROVIDE HISTORICAL ACKNOWLEDGEMENT (MAX : 1 PT)

For All Spaces:

The project or organization develops and implements a comprehensive historical acknowledgment program that meets the following requirements:

- a. Review land deeds, historical records, census data or other credible sources (e.g., articles, websites, surveys) to identify a community that has historically suffered as a result of colonization, displacement and relocation.
- b. Conduct an assessment to determine if any of the organization's current practices or policies are harmful to the community receiving acknowledgment, including, at a minimum, the following:
 1. Discriminatory and harmful labor practices and the implicit biases, policies and structures that lead to them, including those impacting consultants and subcontractors (e.g., collaborative partnerships that may cause representational damage, poor working conditions, participation in modern slavery and privatized prison labor, salary inequity, discriminatory hiring practices and/or lack of representation in leadership).
 2. Occupancy on sacred land (e.g., burial sites, spiritual sites), land protected treaty or agreement (e.g., hunting and gathering rights), or seized land (e.g., by use of eminent domain).
 3. Practices that negatively impact all life and the environment, especially those in closest proximity to the project site (e.g., pollution, deforestation, poor waste management practices).
- c. Create a program for historical acknowledgement that:
 1. Is informed by members and leaders of the community and/or Tribal Nation, as identified through one or more meetings with community representatives.
 2. Acknowledges the beliefs and practices of the community and/or Tribal Nation.
 3. Promotes engagement including reconciliation and/or reparations (as applicable) between the organization and the community and/or Tribal Nation.
 4. Identifies a minimum three-year plan of action for how the organization will develop a meaningful relationship with the community and/or Tribal Nation that will benefit the community (e.g., employment incubator, mental health support, funding agreement, education opportunities, returning land or property).
 5. Includes a signed document acknowledging the partnership demonstrating the program was co-created between the organization and the community and/or Tribal Nation.
- d. Develop a communication plan in a physical or electronic format that provides:
 1. Education for employees and the general public about the community and/or Tribal Nation's historical contribution to society.
 2. Background on the historical acknowledgement program and its development, as well as opportunities for employees to engage with the plan of action.
- e. Express historical context publicly through at least one of the following methods:
 1. Mission statement
 2. Monument
 3. Proclamation
 4. Educational display

5. Plaque
6. Website
7. Restoration of Indigenous names of landmarks
8. Other expressions of historical acknowledgement

Note : This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

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C21 ■ MULTISENSORY DESIGN | O (MAX : 4 PT)

Intent : Support neuro-inclusion through an informed planning process, multisensory design strategies and stakeholder education.

Summary : This WELL feature requires projects to facilitate a stakeholder charrette with a qualified neuro-inclusion professional, implement multisensory design strategies throughout the project and educate employees on neuro-inclusion.

Issue : Neurodiversity refers to the difference in neurocognitive profiles across an entire population.⁴ It is estimated that 15-20% of the world's population exhibits some form of neurodivergence.⁹ Human brains are diverse and vary; therefore, the way people interact with the environment can differ for each person.^{3,5} Neurocognition encompasses a wide range of profiles. This includes neurotypical, which describes someone fitting a majority neurological profile, as well as neurodivergent, which describes someone considered outside a majority or neurotypical neurological profile.^{3,9,20} People who are neurodivergent have unique sensory processing needs and may experience the environment at heightened or lower intensity levels.^{1,2,8} Companies actively hire people who are neurodiverse but few understand how to create supportive spaces.^{3,6,20} Traditional design often overlooks the specific needs of people who are neurodivergent as spaces are usually designed for neurotypical users.^{6,15} This leads to environments that are not supportive of people who are neurodivergent, which may hinder their ability to focus, cause anxiety and limit their participation.^{6,10,13} Although research is limited, there is a growing body of knowledge that is informing neuro-inclusive design.

Solutions : Due to the complex and nuanced nature of creating multisensory design strategies, it is important for qualified neuro-inclusion professionals and people who are neurodivergent to be engaged throughout the integrated planning and design process.^{2,9} Qualified professionals can help identify and implement evidence-based goals and solutions.² People who are neurodiverse can guide design decisions towards those that will be most important, impactful and supportive based on their lived experience.⁹ Creating spaces for neuro-inclusion also requires the implementation of multisensory design strategies to address individual differences in sensory processing. High-stimulation design strategies support hypo-sensitive individuals who may experience understimulation, while low-stimulation design strategies support hyper-sensitive individuals who may experience overstimulation.^{1,8,12} Individual control, access to nature, as well as acoustic and lighting elements are additional factors that support self-regulation and psychological safety for all individuals, regardless of how people process stimuli.^{1,7,8,17} In addition, ensuring that multisensory design strategies are sequenced can help provide natural transitions and a balance of sensory spaces throughout.^{1,11,12,13} Finally, stakeholder education can help communicate the benefits of neuro-inclusive design strategies and policies to all.^{3,4,10}

PART 1 PLAN FOR NEURO-INCLUSION (MAX : 1 PT)

For All Spaces:

Facilitate a stakeholder charrette that meets the following requirements:^{3,9}

- a. Is hosted early in the WELL planning process (may be part of the stakeholder charrette listed in Feature C02 Part 1).
- b. Involves a qualified neuro-inclusion professional.
- c. Includes efforts to involve representatives from the following stakeholders, as applicable:
 1. Owners.
 2. Organizational representatives who identify as neurodivergent.
 3. Interior design professionals (e.g., interior designer, facilities planner, lighting specialist, acoustics specialist).
 4. Human resources.
 5. Facilities managers.
 6. Architects.
 7. Engineers.
 8. Contractors.
- d. Accommodates participation from relevant stakeholders by addressing barriers (e.g., cultural norms or values, literacy levels, language, disabilities, work schedules, childcare) through timing, location, format and communication strategies.⁹
- e. Defines goals for how the project will support neuro-inclusion through multisensory design strategies.
- f. Identifies a list of credible sources (e.g., interviews, research studies, articles) that support how the identified neuro-inclusion goals may be addressed.
- g. Recommends a set of multisensory design strategies to support neuro-inclusion throughout the project.

Note :

This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

PART 2 CREATE SPACES FOR NEURO-INCLUSION (MAX : 2 PT)

For All Spaces:

1: Neuro-inclusive Design

Note : Projects may only achieve this part if Part 1 is also achieved.

The following requirements are met:

- a. At least five of the following multisensory design strategies are implemented:
 1. Adjustable environmental factors (e.g., screens, privacy panels, desks, chairs, lighting, temperature, acoustics) that allow people to control their sensory exposure are incorporated throughout.^{6,15,18}
 2. Natural forms, patterns, lines, shapes and objects (e.g., biomorphic, organic, fractal) are incorporated

throughout.^{5,17,15}

3. Environmental cues (e.g., visual, tactile, auditory, sensory maps) that indicate a shift in behavior or activity between spaces are incorporated throughout.^{2,16,19}
 4. Furniture that allows the body to stay in motion while using (e.g., rocking chair, balance boards).¹⁵
 5. Movable furniture that can be relocated without causing noise disruption (e.g., placed on soft floor, includes furniture pads) is incorporated throughout.⁶
 6. Enclosed areas that have interior windows providing clear lines of sight throughout the space are present.^{6,13,15}
 7. Walls that are curved or have rounded corners are present.^{1,13}
 8. Single-loaded corridors (i.e., doors to rooms are only located on one side only) are present.^{1,13}
 9. Personal storage for each regular occupant is available.¹⁵
 10. At least two entrances to the building are available, one designed with low-stimulation design strategies and one designed with high-stimulation design strategies.
- b. At least four of the following low-stimulation design strategies are implemented in at least two spaces:
1. Environmental factors that support visual, auditory and physical privacy.^{5,11,18}
 2. Areas of refuge.^{1,11,19}
 3. Low-intensity colors and patterns.^{1,5,13}
 4. Low-threshold textural experiences (e.g., materials that are smooth or predictable in typography, carpets with low pile).^{1,11,18}
 5. Restorative elements (e.g., biophilic patterns, calming sounds, horizontal lines).^{13,18,19}
 6. Use of intimate scale (e.g., low ceilings, tight spatial layout).^{6,13,19}
 7. Symmetrical organization of spatial layout and design elements.^{1,13}
- c. At least four of the following high-stimulation design strategies are implemented in at least two spaces:
1. Environmental factors that support visual, auditory and physical stimulation.^{13,15,19}
 2. Active and energizing design elements (e.g., natural sounds, low music, accent lighting).^{6,13,19}
 3. High-intensity colors and patterns.^{6,13,19}
 4. High-threshold textural experiences (e.g., materials that are rough or unpredictable in typography, carpets with high pile).^{6,13,19}
 5. Use of open scale (e.g., high ceilings, open spatial layout).^{6,13,19}
 6. Asymmetrical organization of spatial layout and design elements.^{1,13}
- d. Low-stimulation and high-stimulation spaces are distributed according to the following:
1. Interspersed throughout the project boundary.^{1,13}
 2. Incorporated in both work spaces and non-work spaces.

2: Flexible Work Space Policy

The following requirement is met:

- a. A flexible work space policy is in place that allows employees to work in different spaces within the project boundary and throughout the day based on their individual sensory needs.^{6,10,19}

Note :

This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

PART 3 EDUCATE FOR NEURO-INCLUSION (MAX : 1 PT)

For All Spaces:

1: Neuro-inclusive Employee Communication

The project meets the following requirements:

- a. Identifies and communicates the neuro-inclusive design strategies and related policies to employees through at least two of the following:
 1. Virtual tours, available on-demand.

2. In-person tours, available upon request.
3. Employee handbook.
4. Signage throughout the project boundary.

2: Neurodiversity Education

The project offers educational programming that meets the following requirements:

- a. Is provided by a qualified neuro-inclusion professional.
- b. Is offered annually to all employees.
- c. Is offered live, either in-person or virtually.
- d. Covers the following topics:
 1. Overview of the conditions of people who are neurodivergent.
 2. Preferred terms, language and definitions used to describe people who are neurodivergent.
 3. Evidence-based strategies to support and accommodate neuro-inclusion.
 4. Benefits of neuro-inclusive workplaces.
 5. Evidence-based strategies for preventing, identifying and navigating observed or experienced discrimination toward people who are neurodiverse.

Note :

This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

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APPENDIX C1:

The following topics must be covered by the custom survey selected for Option 2: Custom Survey in Feature C04 Part 1:

1. General building and occupancy information, including job type or time spent in the building.
2. Indoor environmental quality of air, water, light, sound and thermal comfort.
3. Ergonomics, layout and aesthetics.
4. Maintenance and cleanliness.
5. Amenities: access to nature, views and nourishment options.
6. Satisfaction with how policies and amenities impact and support healthy behaviors (e.g., physical activity, healthy eating).
7. Access to and engagement with workplace wellness initiatives or offerings (e.g., physical activity incentive programs, health benefits and services).
8. Employee support policies (e.g., paid parental and family leave, flexible working arrangements).
9. Productivity and engagement (e.g., through measures of hours worked or motivation).
10. Self-rated health and well-being.
11. Standard sociodemographic information (age and gender at minimum).

APPENDIX C2:

Approved additional topics to add to the pre-approved survey in Part 1: Select Enhanced Survey in Feature C05: Enhanced Occupant Survey.

Category	Topic
Healthy Behaviors:	Mode of transportation to and from work and distance or time traveled
	Hydration
	Sleep satisfaction, quality and/or quantity
	Physical activity
	Alcohol consumption
	Healthy eating
	Ability to take restorative breaks
	Smoking habits
Enhanced Health and Well-being:	Sick building syndrome
	Mental health
	Social, cultural or economic well-being
	Musculoskeletal issues (e.g., back, neck pain)
	Health literacy
Performance and Resilience:	Assessment of individual work style, patterns, processes, space utilization and ability to focus or collaborate
	Workplace performance
	Engagement
	Workload, stress, burnout and/or employee resilience
	Creative thinking
	Safety and security, including for diverse population groups (e.g., cultural, ethnic, gender, ability, age)
Policies and Culture:	Emergency preparedness (e.g., pandemic, fire, natural disaster)
	Workplace wellness programs and perceived effectiveness
	Leadership investment in employee health and perceived effectiveness
	Social equity programs and perceived effectiveness
	Comparison to previous space
Other:	Values related to, level of access to and experience of nature
	Feedback on specific design interventions
	Healthy behaviors, ergonomics, mental health and productivity for remote workers
	Additional sociodemographic information (e.g., education, ethnicity, income)

APPENDIX C3:

The following supplies must be included in first aid kits per the requirements of Feature C14 Emergency Resources:¹

Bandages

- Adhesive bandage
- Adhesive tape
- Burn dressing (gel soaked)
- CPR breathing barrier
- Eye covering (with means of attachment)
- Roller bandage
- Sterile pad
- Trauma pad
- Triangular bandage (e.g., for a sling)

Lotions, oils and other fluids

- Antibiotic application
- Antiseptic
- Burn treatment
- Eye wash
- Hand sanitizer

First aid tools

- Cold pack
- First aid guide
- Medical exam gloves
- Scissors

References:

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